



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض ربيعاً : حبيب محمد يحيى
المحافظة : القاهرة
الحى / المدينة : حلوان
رقم البطاقة : 28702170102886 رقم التليفون : 01124503256
العمر (تاريخ الميلاد) : 17-02-1987 المهنة : ربه منزل
العنوان : 1 عرب كفر العلو
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة
قسم العلاج :
الأورام

[11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: حبيب محمد يحيى يحيى
National ID: 28702170102886



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف شخصية



حبيبه
محمد يحيى عبدالمطلب
١ عرب كفر العلو
حلوان - القاهرة

٢٨٧٠٢١٧٠١٠٢٨٨٦
٢٨٧٠٢١٧٠١٠٢٨٨٦

KP8271122

٢٠٢٤/٠٦ ٢٨٧٠٢١٧٠١٠٢٨٨٦

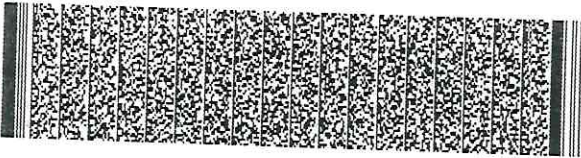


بدون عمل

أنثى
مسلمة
متزوجة
محمد حسن أحمد محمد

البطاقة سارية حتى ٢٠٣١/٠٦/٢٦

١



Patient ID: 97251

Age: ٢٩ سنة

Patient Name English: Habiba Mohamed yhia

2025/07/14 01:27:24 PM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: left breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast (ACR c).
- UOQ posterior third partially obscured density
- No distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated 10 o'clock, zone b cyst measures 8 x 6 mm , likely correspond to aforementioned mammographic finding.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Left breast

- Heterogeneously dense breast (ACR c), with relatively small size breast, and diffuse skin thickening.
- Large retro areolar irregular spiculated dense mass lesion.
- Ultrasound demonstrated a large irregular ill-defined hypoechoic mass lesion, seen at retro areolar region extending mainly at UOQ, zone a& b, with moderate color uptake on Doppler study
- Hyperechoic fat lobules noted mainly at LOQ
- Mild skin thickening.
- Pathological level I & II axillary LNs, largest noted at level I , MEASURES 28 x 15 mm , lobulated ? Amalgamated lymph nodes .

Impression:

Left breast (BIRADS 5): locally advanced? Malignant process with ipsilateral pathological axillary lymph nodes.

Right breast (BIRADS 2): 10 o'clock cyst.

Recommendations:

For left breast sonar guided biopsy.

Dr.RANIA AHMED, MD

9/17/26, 12:23 PM

357882|63611



Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناتيونال - مصر - معمل الباثولوجي



Patient Name	حبيبہ محمد رجا	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Five grey, white tissue cores, the largest one measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9), E-cadherin & SISH.

Microscopic examination:

- Histologic type: Mixed invasive duct & invasive lobular carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent.
- LCIS: Absent.
- Microcalcification: Absent.
- Angiolinvasion: Absent.
- Perineural invasion: Absent
- Necrosis: Focal.

Results:

* IPT (All controls showed appropriate reactivity):

- E-cadherin: Widely positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).
- Ki67%: 40%.

Conclusion:

Left breast mass, Tru-cut biopsy:



مركز قبة زرقاء (4) - الشور الأول - مبنى زرقاء (987) - مركز تفتيش النيل - معمل الباثولوجي - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche- Most El Qadema - Cairo, Egypt

Patient Name	حبيبة محمد يحيى	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

- Mixed invasive duct & invasive lobular carcinoma, Grade 3.

IPt:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified. Group II.
- (Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).
- Ki67%: 40%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



جوستاف روسي القى فطرنال - مصر عمل الباثولوجى
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient ID: 97251	Patient Name English: Hejira Mohamed Yha
Dr. Y. A. T. Age:	2025/07/14 01:27:24 PM Report Date

Women's Health Outreach Program
Bilateral Full Field Digital Mammography

Clinical summary: left breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- o Heterogeneously dense breast (ACR c).
- o UOQ posterior third partially obscured density
- o No distortion or suspicious clusters of micro calcifications.
- o Ultrasound demonstrated 10 o'clock, zone b cyst measures 8 x 6 mm, likely correspond to aforementioned mammographic finding.
- o Normal skin and nipple/areolar complex.
- o Preserved axillary LNs.

Left breast

- o Heterogeneously dense breast (ACR c), with relatively small size breast, and diffuse skin thickening.
- o Large retro areolar irregular spiculated dense mass lesion.
- o Ultrasound demonstrated a large irregular ill-defined hypoechoic mass lesion, seen at retro areolar region extending mainly at UOQ, zone a& b, with moderate color uptake on Doppler study.
- o Hyperechoic fat lobules noted mainly at LOQ
- o Mild skin thickening.
- o Pathological level I & II axillary LNs, largest noted at level I, MEASURES 28 x 15 mm, lobulated? Amalgamated lymph nodes.

Impression:

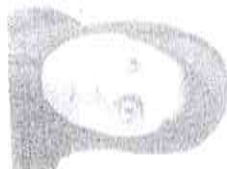
Left breast: (BIRADS 5): locally advanced? Malignant process with ipsilateral pathological axillary lymph nodes.

Right breast: (BIRADS 2): 10 o'clock cyst.

Recommendations:

For left breast sonar guided biopsy.

Dr. RANIA AHMED, MD



جنسية
محمد يحيى يحيى عبدالمطلب
١ عرب كفر الشيخ
طوان - القاهرة



٢٨٧٠٢١٧٠١٠٢٨٨٦

KP8271122



جنسية
محمد يحيى احمد محمد
الطائفة مصرية حتى ٢٠٣٧/١/٢٧

٢٠١٦/١٠ ٢٨٧٠٢١٧٠١٠٢٨٨٦
٠٥٩٩



Patient Name	حبيبہ محمد رجا	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Five grey, white tissue cores, the largest one measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9), E-cadherin & SISH.

Microscopic examination:

- Histologic type: Mixed invasive duct & invasive lobular carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent.
- LCIS: Absent.
- Microcalcification: Absent.
- Angiolinvasion: Absent.
- Perineural invasion: Absent
- Necrosis: Focal.

Results:

* IPT (All controls showed appropriate reactivity):

- E- cadherin: Widely positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).
- Ki67%: 40%.

Conclusion:

Left breast mass, Tru-cut biopsy:



Patient Name	حبيبة محمد يحيى	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

- Mixed invasive duct & invasive lobular carcinoma, Grade 3.

IPt:

- ER: Positive (score 8/8).

- PR: Positive (score 8/8).

- Her2: Equivocal (score 2+).

- SISH: Not amplified. Group II.

(Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).

- Ki67%: 40%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+ : as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+ : based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+ : based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



مركز الباثولوجى - مصر القى فطوئال - جوستاف روسي (987) - جوستاف روسي القى فطوئال - مصر القى فطوئال - جوستاف روسي
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

12:23 PM

357882|6381|

Gustave Roussy International-Egypt- Pathology Lab
جورستاف رويسى القى زلقوئال - مصر -مصر -مصر

Patient Name	جيبه محمد يحيى	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

خبرية رقم (4) - (5) نور القبول - مصرى رقم (987) - نور القبول - مصرى القيدية - (1) القبول - مصرى
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Maser El Qadema - Cairo, Egypt

Patient ID: 97251
JCA V. ALI, FRCR

Patient Name: Engish, Hejira Mohamed Yha
025/07/14 01:27:24 PM Report Date

Women's Health Outreach Program
Bilateral Full Field Digital Mammography

Clinical summary: left breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- o Heterogeneously dense breast (ACR c).
- o UOQ posterior third partially obscured density
- o No distortion or suspicious clusters of micro calcifications.
- o Ultrasound demonstrated 10 o'clock, zone b cyst measures 8 x 6 mm, likely correspond to aforementioned mammographic finding.
- o Normal skin and nipple/areolar complex.
- o Preserved axillary LNs.

Left breast

- o Heterogeneously dense breast (ACR c), with relatively small size breast, and diffuse skin thickening.
- o Large retro areolar irregular spiculated dense mass lesion.
- o Ultrasound demonstrated a large irregular ill-defined hypoechoic mass lesion, seen at retro areolar region extending mainly at UOQ, zone a& b, with moderate color uptake on Doppler study.
- o Hyperechoic fat lobules noted mainly at LOQ
- o Mild skin thickening.
- o Pathological level I & II axillary LNs, largest noted at level I, MEASURES 28 x 15 mm, lobulated? Amalgamated lymph nodes.

Impression:

Left breast: (BIRADS 5): locally advanced? Malignant process with ipsilateral pathological axillary lymph nodes.

Right breast: (BIRADS 2): 10 o'clock cyst.

Recommendations:

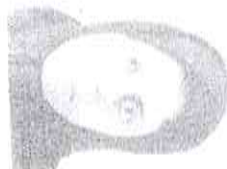
For left breast: sonar guided biopsy.

Dr. RANIA AHMED, MD

الخطوب

برنت تا صبح صبح

مهمه عماره العراش بالعمريه



جنسية
محمد يحيى يحيى عبدالمطلب
١ عرب كفر الشيخ
طوان - القاهرة



٢٨٧٠٢١٧٠١٠٢٨٨٦

KP8271122



جنسية
محمد يحيى احمد محمد
الطائفة مصرية حتى ٢٠٣٧/١/٢٧

٢٠١٦/١٠ ٢٨٧٠٢١٧٠١٠٢٨٨٦
٠٥٩٩



Patient Name	حبيبہ محمد رجا	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Five grey, white tissue cores, the largest one measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9), E-cadherin & SISH.

Microscopic examination:

- Histologic type: Mixed invasive duct & invasive lobular carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent.
- LCIS: Absent.
- Microcalcification: Absent.
- Angiolinvasion: Absent.
- Perineural invasion: Absent
- Necrosis: Focal.

Results:

* IPT (All controls showed appropriate reactivity):

- E- cadherin: Widely positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).
- Ki67%: 40%.

Conclusion:

Left breast mass, Tru-cut biopsy:



Patient Name	حبيبة محمد يحيى	Case No.	357882
Sex/Age	Female 39 Y	Patient No.	25639
Registered Date	13/8/2026 11:19:43 AM	Referred By Dr.	Interventional Radiology
Validation Date	06/9/2026 10:23:42 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 12:23:22 PM	Pathology No.	2494/26

- Mixed invasive duct & invasive lobular carcinoma, Grade 3.

IPt:

- ER: Positive (score 8/8).

- PR: Positive (score 8/8).

- Her2: Equivocal (score 2+).

- SISH: Not amplified. Group II.

(Average Her2 copy No.: (2.25), Her2/CEP17 ratio: (2.1)).

- Ki67%: 40%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+ : as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+ : based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+ : based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



قسم الباثولوجى - مستشفى جوستاف روسي القى فطوئال - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qaderna - Cairo, Egypt

Patient ID: 97251
JCA V. ALI, FRCR

Patient Name: Engish, Hejira Mohamed Yha
025/07/14 01:27:24 PM Report Date

Women's Health Outreach Program
Bilateral Full Field Digital Mammography

Clinical summary: left breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- o Heterogeneously dense breast (ACR c).
- o UOQ posterior third partially obscured density
- o No distortion or suspicious clusters of micro calcifications.
- o Ultrasound demonstrated 10 o'clock, zone b cyst measures 8 x 6 mm, likely correspond to aforementioned mammographic finding.
- o Normal skin and nipple/areolar complex.
- o Preserved axillary LNs.

Left breast

- o Heterogeneously dense breast (ACR c), with relatively small size breast, and diffuse skin thickening.
- o Large retro areolar irregular spiculated dense mass lesion.
- o Ultrasound demonstrated a large irregular ill-defined hypoechoic mass lesion, seen at retro areolar region extending mainly at UOQ, zone a& b, with moderate color uptake on Doppler study.
- o Hyperechoic fat lobules noted mainly at LOQ
- o Mild skin thickening.
- o Pathological level I & II axillary LNs, largest noted at level I, MEASURES 28 x 15 mm, lobulated? Amalgamated lymph nodes.

Impression:

Left breast: (BIRADS 5): locally advanced? Malignant process with ipsilateral pathological axillary lymph nodes.

Right breast: (BIRADS 2): 10 o'clock cyst.

Recommendations:

For left breast sonar guided biopsy.

Dr. RANIA AHMED, MD

الخطوب

برنت تا صبح صبح

مهمه عماره العراش بالعمريه